



Common Psychiatric Diseases

Learning Objectives

- ⇒ To learn the clinical features of depression, anxiety neurosis, epilepsy (non-organic)
- ⇒ To differentiate depression, anxiety neurosis, epilepsy (non-organic)
- ⇒ To perform mental status examination for depression, anxiety neurosis, epilepsy (non-organic)

Depression

Depressive disorders are characterized by sadness severe enough or persistent enough to interfere with function and often decreased interest or pleasure in activities. Exact cause is unknown but probably involves heredity, changes in neurotransmitter levels, altered neuroendocrine functions, and psychosocial factors. Diagnosis is based on history.

- ⊛ The term depression is often used to refer to any of several depressive disorders.



Figure 220: Clinical features of depression

- ⊛ 3 types are mentioned in DSM - IV (Diagnostic and statistical manual of mental disorders - 4th edition) by specific symptoms:

1. Major depressive disorder (often called major depression)
2. Dysthymia
3. Depressive disorder not otherwise specified

⊛ 2 others are classified by aetiology:

1. Depressive disorder due to a general physical conditions
 2. Substance induced depressive disorder
- ⊛ Depressive disorders occur at any age, but typically develop during mid teens, 20s or 30s. In primary care settings, as many as 30% of the patients report depressive symptoms, but <10% have major depression.
 - ⊛ The term depression is often used to describe the low or discouraged mood that results from disappointments or losses. Here, the better term could be demoralization. Low mood usually lasts days rather than months.
 - ⊛ Exact cause is unknown. Major life stresses and losses commonly precede episodes of depression.
 - ⊛ Depression causes cognitive, psychomotor and other types of dysfunction (e.g. poor concentration, fatigue, loss of sexual desire, loss of pleasure), as well as depressed mood. Other mental symptoms (anxiety attacks and panic attacks) commonly coexist, sometimes complicating the diagnosis and treatment.
 - ⊛ Patients with all forms of depression may have substance abuses more commonly to ethyl alcohol.

Major depression (unipolar disorder)

Periods that include ≥ 5 mental or physical symptoms

and last ≥ 2 weeks are classified as major depression. One of the symptoms must be sadness deep enough to be described as despondency or despair or loss of interest or pleasure in usual activities (anhedonia). Other mental symptoms include feelings of worthlessness or guilt, recurrent thoughts of death or suicide, and reduced ability to concentrate. Physical symptoms include changes in weight or appetite, loss of energy, fatigue, psychomotor retardation and sleep disorders (insomnia or hypersomnia, early morning awakening).

- Patients may appear miserable with tearful eyes, furrowed brows, down turned corners of the mouth, slumped posture, poor eye contact, lack of facial expression, little body movement, speech changes (soft voice, lack of prosody, use of monosyllable words).

• **4 subgroups are made:** Psychotic, catatonic, melancholic and atypical.

1. **Psychotic:** This subgroup is characterized by delusions, often having committed unpardonable sins or crimes, of harbouring incurable or shameful disorders or of being persecuted. Patients with delusions may also have auditory or visual hallucinations (e.g. hearing accusatory or condemning voices). If only voices are described, careful consideration should be given to whether the voices represent true hallucinations.
2. **Catatonic:** This subgroup is characterized by severe psychomotor retardation or excessive purposeless activity, withdrawal and in some patients, grimacing and mimicry of speech (echolalia) or movement (echopraxia).
3. **Melancholic:** This subgroup is characterized by loss of pleasure in nearly all activities, inability to respond pleasurable stimuli, unchanging emotional expression, excessive or inappropriate guilt, early morning awakening, marked psychomotor retardation and significant anorexia or weight loss.
4. **Atypical:** This subgroup is characterized by a brightened mood in response to positive events and rejection sensitivity resulting in depressed overreaction to perceived rejection or criticism, feelings of leaden paralysis or anergy, weight gain or increased appetite, and hypersomnia.

Dysthymia

- Low level subthreshold depressive symptoms that persist for ≥ 2 years are classified as dysthymia.
- Symptoms typically begin insidiously during adolescence and follow low grade course over many years or decades.
- Affected patients may have gloomy, pessimistic,

humourless, passive, lethargic, introverted, hypercritical of self or others; and complaining.

- Patients with chronic depressive states; whether dysthymia or chronic major depression are also more likely to have underlying anxiety, substance use or personality disorders.

Depression not otherwise specified (NOS)

- Cluster of symptoms that do not meet criteria for other depressive disorders are classified as depression NOS. for example, minor depression may involve ≥ 2 weeks of any of the symptoms of major depression, but fewer symptoms than the 5 are required for diagnosing major depression.
- Brief depressive disorder involves the same symptoms required for diagnosing major depression, but lasts for 2 days to 2 weeks.
- Diagnosis by DSM – IV clinical criteria. No laboratory findings are pathognomonic for depressive disorders.

Anxiety Neurosis

The concept of anxiety neurosis was based on Freud's early theoretical description of free floating anxiety and the concept included both panic disorder and GAD (Generalised Anxiety Disorder) concurrently. Anxiety neurosis term was coined by Sigmund Freud a century ago. Based on this description, we are going to analyse two descriptions here: GAD (Generalized Anxiety Disorder) and panic disorder.



Figure 221: Anxiety neurosis

Generalised Anxiety Disorder (GAD)

This disease is characterized by excessive, almost daily

anxiety and worry for ≥ 6 months about many activities or events. The cause is unknown, although it commonly coexists in people who have alcohol abuse, major depression or panic disorder. Diagnosis is based on history and physical examination.

- GAD is common, affecting 1% of the population within 1 year period. Women are twice as likely to be affected as men. The disorder often begins in childhood or adolescence, but may begin at any age.
- The patient will have multiple worries, which often shift over time. Common worries include work and family responsibilities, money, health, safety, car repairs and chores.
- The course is usually fluctuating and chronic, with worsening during stress. Most people with GAD have one or more other comorbid psychiatric disorders, including major depression, specific phobia, social phobia and panic disorder.
- **Diagnosis¹** is based on clinical criteria of DSM – IV.

Panic disorder

Panic disorder is occurrence of repeated panic attacks typically accompanied by fears about future attacks or changes in behaviour to avoid situations that might predispose to attacks. Panic attack is the sudden onset of a discrete, brief period of intense discomfort, anxiety or fear accompanied by somatic or cognitive symptoms. Diagnosis is clinical.

- Panic attacks are common, affecting as many as 10% of the population in a single year. Most people recover without treatment; a few develop panic disorder. Panic disorder is uncommon, affecting 2 – 3% of the

population in 12 months period.

- Panic disorder usually begins in late adolescence or early adulthood and affects women 2 to 3 times more often than men.
- **A panic attack²** involves the sudden onset of at least 4 of 13 symptoms. Symptoms usually peak within 10 minutes, thereafter leaving little time for a physician to observe.
- Panic attacks may occur in any anxiety disorder, usually in situations tied to the core features of the disorder (e.g. a person with a phobia of snakes may panic at seeing a snake).
- Most people with panic disorder anticipate and worry about another attack (anticipatory anxiety) and avoid places or situations where they previously have panicked.

People with panic disorder often worry that they have a dangerous heart, lung or brain disorder and repeatedly visit their family physician or emergency department seeking help.

- Many people of panic disorder also have symptoms of depression.
- Diagnosis is done after physical disorders that can mimic anxiety are eliminated and symptoms meet diagnostic criteria as per DSM IV.

Epilepsy (Non-Organic)

Seizure disorders are group of disorders where abnormal, unregulated electrical discharge that occurs within the brain's cortical gray matter and transiently interrupts normal brain function. A seizure typically

- 1 With the exclusion of other causes, affirmative answers to the following questions confirm the diagnosis.

- Does the patient have excessive, almost daily anxiety and worry about many activities or events?
- Have the anxiety or worry lasted for ≥ 6 months?
- Does the patient have difficulty in controlling worry?
- Does the patient also have ≥ 3 of the following symptoms?
 - o Restlessness
 - o Unusual fatigability
 - o Difficulty concentrating
 - o Irritability
 - o Muscle tension
 - o Disturbed sleep
- Have at least some of these symptoms been present for a majority of days in the past 6 months?
- Is the focus of worry broader than that in other anxiety disorders?
- Have symptoms caused substantial distress or interfered with function?

- 2 Symptoms of panic attack:

Cognitive:

- Fear of dying
- Fear of going crazy or of losing control
- Feelings of unreality, strangeness or detachment from self (depersonalization)

Somatic:

- Chest pain or discomfort
- Dizziness, unsteady feelings or faintness
- Feeling of choking
- Flashes or chills
- Nausea or abdominal distress
- Numbness or tingling sensations
- Palpitations or accelerated heart rate
- Sensations of shortness of breathlessness or smothering feeling
- Trembling or shaking

produces altered awareness, abnormal sensations, focal involuntary movements or convulsions (widespread violent involuntary contraction of voluntary muscles).

- ⊗ Epilepsy (also called epileptic seizure disorder) is a chronic brain disorder characterized by recurrent (≥ 2), unprovoked seizures (i.e. not related to reversible stressors).
- ⊗ In non organic epilepsy, various brain disorders such as malformations, strokes, brain tumours, CNS infections, cardiovascular disorders, are not identified after elaborate patient history analysis.

- ⊗ Functional disturbances which can occur because of metabolic disturbances such as hypocalcemia or hypoglycemia or hyponatremia; drug toxicity or drug withdrawal; alcohol withdrawal can lead to manifestation of attacks of epilepsy. These causes are categorized under non organic causes of episodes of epilepsy.

- ⊗ Diagnosis can be done by elaborate clinical evaluation and appropriate laboratory investigations. For example, when one suspects hypoglycemia as the cause of epileptic attack, then blood sugar analysis should be done to rule out the cause of epilepsy.

Key summary of the chapter

- ⊗ Common psychiatric diseases which may afflict human beings include depression, anxiety neurosis. In the patients of epilepsy due to non organic causes, abnormal functioning of the nervous system is observed.
- ⊗ Depressive disorders are characterized by sadness enough or persistent enough to interfere with function and often decreased interest or pleasure in activities. Exact cause is unknown, but probably involves heredity, changes in neurotransmitter levels, altered neuroendocrine functions, and psychosocial factors. Diagnosis is based on elaborate case history analysis.
- ⊗ Major depression is often used to refer to any of several depressive disorders. 3 types are mentioned: Major depressive disorder, dysthymia and depressive disorder not otherwise specified. Based on the aetiology, 2 types of depression are described: depressive disorder due to a physical condition & substance induced depressive disorder.
- ⊗ Anxiety neurosis is now known as generalized anxiety disorder. Panic disorder is also having frequent episodes of panic attacks where the person will have anxiety.
- ⊗ Epilepsy of non-organic origin refers to the clinical condition where the person may have seizure episodes, but structural abnormalities in the body is not identifiable by extensive imaging techniques.

MCQs

1. The duration of the illness in the patient of dysthymia is upto or more thanyear/s.

- a. 1 year;
- b. 2 years;
- c. 3 years;
- d. 4 years

Answer: (b)

2. Melancholic is a type of

- a. Major depression;
- b. Mania;
- c. Mental retardation;
- d. Mental agitation

Answer: (a)

3. The term anxiety neurosis is coined by

- a. Sigmund Freud;
- b. Eric Burne;
- c. Marie Curie;
- d. William Harvey

Answer: (a)

4. Generalised anxiety disorder (GAD) is more commonly seen in

- a. Males;
- b. Females;
- c. Equally in males & females
- d. Children

Answer: (b)

5. DSM-IV is related to

- a. Psychiatric diseases;
- b. Neurological diseases;

- c. Alimentary canal diseases;
- d. Cardiac diseases

Answer: (a)

Chapter resources

- ⊗ https://www.freepik.com/free-vector/anxiety-concept-illustration_21864011.htm#query=anxiety%20disorder&position=2&from_view=keyword&track=ais&uuiid=2d4a7754-e8c6-4366-87bd-4787ccfc17b2
- ⊗ Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- ⊗ The Merck Manual of Diagnosis and Therapy by Robert S Porter and Justin L Kaplan, 19th edition, Merck Sharp & Dohme Corp, New Jersey, 2011
- ⊗ Chamberlain's symptoms and signs in clinical medicine by Colin Ogilvie and Christopher C Evans, 12th edition, Butterworth-Heinemann Publishers, 1997
- ⊗ Best & Taylor's physiological basis of medical practice by O P Tandon and Y Tripathi, 13th edition, Wolters Kluwer Health (India) Pvt. Ltd, Gurgaon, 2012
- ⊗ Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006
- ⊗ www.medium.com, www.sciencedirect.com